



Bill Of Supply

Name : Mrs. K MEGHANA
Age/Gender : 28 Y F
Contact No : +918332808860
Address : BANGLORE
UHID : RMAR.0000072295

Bill No : RMAR-OCS-820013
Bill/Reg Date : 10.07.2026 08:48
Referred by : Dr. GARIMA JAIN
Center : Cradle Marathahalli
Emp No/Auth Code :

#	Department	Description Of Service	SAC/HSN Code	Qty	Rate	Gross Value	Discount	CGST Rate	CGST Amt	SGST/UTGST Rate	SGST/UTGST Amt	Net Value
1	Lab Tests	COMPLETE BLOOD COUNT (CBC)	999316	1	418.00	418.00	41.80	0.00	0.00	0.00	0.00	376.20
2	Lab Tests	Prothrombin Time (PT/INR)	999316	1	632.50	632.50	63.25	0.00	0.00	0.00	0.00	569.25
3	Lab Tests	THYROID STIMULATING HORMONE (TSH), SERUM	999316	1	517.00	517.00	51.70	0.00	0.00	0.00	0.00	465.30
4	Lab Tests	HBS AG SCREENING(RAPID)	999316	1	1,014.00	1,014.00	101.40	0.00	0.00	0.00	0.00	912.60
5	Lab Tests	HIV RAPID	999316	1	748.00	748.00	74.80	0.00	0.00	0.00	0.00	673.20

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APOLLO SPECIALTY HOSPITALS PRIVATE LIMITED
(Formerly known as Nova Specialty Hospitals Private Limited)
CIN - U85100TG2009PTC099414
Regd. Office: 1-10-60/62, Ashoka Raghupathi Chambers, 5th Floor, Begumpet, Hyderabad - 500 016, Telangana

GSTIN: 29AADCN1803G1ZP

Address: Opus 143, 1st Cross, 5th Block, Koramangala, Bengaluru, Karnataka

6	Lab Tests	COMPLETE URINE EXAMINATION	999316	1	403.00	403.00	40.30	0.00	0.00	0.00	0.00	362.70
7	Round Off			1	-0.25	-0.25	0.00	0.00	0.00	0.00	0.00	-0.25

Payments	Receipt No	Mode	Amount
Settlement			
10-07-2026	RMARRCA659454	PhonePe	3,359.00

Bill Amount: 3,732.25

Total Discount: 373.25

Patient Payment: 3,359.00

Patient Due: 0.00

Received with thanks: Three Thousand Three Hundred Fifty Nine Rupees only

Authorized Signature : (Arjita Biswas)

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Patient Name : Mrs.K MEGHANA
Age/Gender : 28 Y 7 M 24 D/F
UHID/MR No : RMAR.0000072295
Visit ID : RMAROPV822305
Ref Doctor : Dr.Dr GARIMA JAIN

Collected : 10/Jul/2026 08:53AM
Received : 10/Jul/2026 09:16AM
Reported : 10/Jul/2026 09:28AM
Status : Final Report

DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE BLOOD COUNT (CBC) , WHOLE BLOOD EDTA				
HAEMOGLOBIN	12.0	g/dL	12-15	Spectrophotometer
PCV	34.60	%	36-46	Electronic pulse & Calculation
RBC COUNT	4.26	Million/cu.mm	3.8-4.8	Electrical Impedance
MCV	81.3	fL	83-101	Calculated
MCH	28.1	pg	27-32	Calculated
MCHC	34.6	g/dL	31.5-34.5	Calculated
R.D.W	14.3	%	11.6-14	Calculated
TOTAL LEUCOCYTE COUNT (TLC)	13,730	cells/cu.mm	4000-10000	Electrical Impedance
DIFFERENTIAL LEUCOCYTIC COUNT (DLC)				
NEUTROPHILS	73	%	40-80	Flow cytometry
LYMPHOCYTES	22	%	20-40	Flow cytometry
EOSINOPHILS	1	%	1-6	Flow cytometry
MONOCYTES	4	%	2-10	Flow cytometry
BASOPHILS	0	%	0-2	Flow cytometry
CORRECTED TLC	13,730	Cells/cu.mm		Calculated
ABSOLUTE LEUCOCYTE COUNT				
NEUTROPHILS	10022.9	Cells/cu.mm	2000-7000	Calculated
LYMPHOCYTES	3020.6	Cells/cu.mm	1000-3000	Calculated
EOSINOPHILS	137.3	Cells/cu.mm	20-500	Calculated
MONOCYTES	549.2	Cells/cu.mm	200-1000	Calculated
Neutrophil lymphocyte ratio (NLR)	3.32		0.78- 3.53	Calculated
PLATELET COUNT	329000	cells/cu.mm	150000-410000	Electrical impedance
MPV	8	fL	8.1-13.9	Calculated

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Collected : 10/Jul/2026 08:53AM
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Reported : 10/Jul/2026 01:09PM
Status : Final Report

DEPARTMENT OF COAGULATION

Test Name	Result	Unit	Bio. Ref. Interval	Method
PROTHROMBIN TIME (PT/INR) , SODIUM CITRATE PLASMA				
Prothrombin Time	15.0	Seconds	11.6 - 16.6	Optomechanical clot detection
Control (MNPT)	14.40	Seconds		Optomechanical clot detection
Ratio	1.04			Calculated
Prothrombin Index	96.00	%		Calculated
International Normalized Ratio (INR)	1.05			Calculated

Comment:

- Prothrombin time measures the extrinsic coagulation pathway which consists of activated Factor VII (VIIa), Tissue factor and Proteins of the common pathway (Factors X, V, II & Fibrinogen). This assay is used to control long term oral anticoagulant therapy, evaluation of liver function & to evaluate coagulation disorders.
- INR is the parameter of choice in monitoring adequacy of oral anticoagulant therapy. Marked elevation of INR in patients receiving oral anticoagulant therapy is a marker of excessive anticoagulation and requires prompt action.

REFERENCE GROUP	INTERNATIONAL NORMALIZED RATIO (INR)
NORMAL POPULATION	0.9 – 1.1
PATIENTS ON ANTICOAGULANT THERAPY	
STANDARD DOSE THERAPY	2.0 – 3.0
HIGH DOSE THERAPY	2.5 – 4.5

- Factors which can affect the prothrombin time results which need to be considered appropriately during interpretation are –
- Preanalytical – appropriate sample collection preferably during basal level (half an hour before next dose of oral anticoagulant), with good phlebotomy practices, timely centrifugation & separation of citrated plasma, sample transported in required conditions. Moderate Lipemic, hemolyzed samples can give lower results than expected.
- Interactions - Some medications (antibiotics-co-trimoxazole, Metronidazole, Barbiturates, anti-histaminics (cimetidine), OC pills, antiepileptics, digoxin, heparin), food (containing vit K like broccoli, chick peas, soyabean, asparagus, pork/beef liver) and alcohol may affect test results.
- Indirect inhibitors of thrombin and/or factor Xa, namely, low-molecular weight heparin (LMWH), fondaparinux- have no significant effect on PT, while, direct thrombin inhibitors, (bivalirudin, argatroban and dabigatran)- prolong the PT only moderately and therefore PT is not suitable for assessment of these drugs.
- PT results obtained by different reagent/analyzer combinations will differ due to difference in testing methodology. Hence it is advisable to follow up with one laboratory only.

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DEPARTMENT OF IMMUNOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
TSH (Ultrasensitive/4thGen) , SERUM	3.960	µIU/mL	0.270-4.20	ECLIA

Comment:

TSH is a glycoprotein hormone secreted by the anterior pituitary. TSH is a labile hormone & is secreted in a pulsatile manner throughout the day and is subject to several non-thyroidal pituitary influences. Significant variations in TSH can occur with circadian rhythm, hormonal status, stress, sleep deprivation, caloric intake, medication & circulating antibodies. It is important to confirm any TSH abnormality in a fresh specimen drawn after ~ 3 weeks before assigning a diagnosis, as the cause of an isolated TSH abnormality.

For pregnant females	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 – 3.0
Third trimester	0.3 – 3.0

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DEPARTMENT OF CLINICAL PATHOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE URINE EXAMINATION (CUE) , URINE				
Physical Examination				
COLOUR	PALE YELLOW		PALE YELLOW	Visual
TRANSPARENCY	CLEAR		CLEAR	Physical Measurement
pH	6.00		5-7.5	Double Indicator
SP. GRAVITY	1.010		1.002-1.030	Protein Error of Indicator
BIOCHEMICAL EXAMINATION				
URINE PROTEIN	NEGATIVE		NEGATIVE	Protein Error of Indicator/Sulpho salicylic Acid
GLUCOSE	NEGATIVE		NEGATIVE	Glucose oxidase/Benedicts Test
URINE BILIRUBIN	NEGATIVE		NEGATIVE	AZO coupling reaction/Fouchets
URINE KETONES (RANDOM)	NEGATIVE		NEGATIVE	Sodium nitro prusside/Rotheras Test
UROBILINOGEN	NORMAL		NORMAL	Modified Ehrlichs
NITRITE	NEGATIVE		NEGATIVE	Diazotization
LEUCOCYTE ESTERASE	NEGATIVE		NEGATIVE	Leucocyte Esterase
CENTRIFUGED SEDIMENT WET MOUNT AND MICROSCOPY				
Pus Cells	2-3	/hpf	0-5	Microscopy
EPITHELIAL CELLS	2-3	/hpf	<10	Microscopy
RBC	NIL	/hpf	0-2	Microscopy
CASTS	NIL		0-2 Hyaline Cast	Microscopy
CRYSTALS	ABSENT		ABSENT	Microscopy

Comment:

All urine samples are checked for adequacy and suitability before examination. All abnormal chemical examination are rechecked and verified by manual methods. Microscopy findings are reported as an average of 10 high power fields.

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DEPARTMENT OF SEROLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
HBS AG SCREENING(RAPID) , SERUM	NON REACTIVE		NON REACTIVE	ICT

Comment:

Interpretation:

- This is a screening assay which detects the first serological marker of Hepatitis B as early as 4-16 weeks after exposure.
- It persists during acute illness and usually disappears 12-20 weeks after onset of symptoms. Persistence of HBsAg for more than 6 months indicates development of carrier state or chronic liver disease
- A negative test result does not exclude with certainty a possible exposure to or an infection with the hepatitis B virus.
- It is recommended that a positive result of HBsAg must be confirmed using a different enzyme immunoassay kit or by using a confirmatory assay based on neutralisation with human anti hepatitis B surface antibody and/or HBV PCR
- Based upon clinical history it may become necessary to test for presence of other markers of hepatitis B virus infection.

Test Name	Result	Unit	Bio. Ref. Interval	Method
HIV RAPID , SERUM	NON REACTIVE		NON REACTIVE	ICT

Comment:

This test is used for screening for HIV-1 and/or HIV-2 infection in asymptomatic patients. Negative HIV-1/-2 antigen and antibody screening test results usually indicate absence of HIV-1 and HIV-2 infection. Reactive HIV-1/-2 antibody screening test results suggest the presence of HIV-1 and/or HIV-2 infection, but it is not diagnostic for HIV infection. All samples detected reactive must be confirmed by using HIV Western Blot.

Some samples show cross reactivity for HIV antibodies. Following factors are found to cause false positive HIV antibody test results: Naturally occurring antibodies, Passive immunization, Leprosy, Renal Disorders, Tuberculosis, Myco-bacterium avium, Herpes simplex, Hypergammaglobulinemia, Malignant neoplasms, Rheumatoid arthritis, Tetanus vaccination, Autoimmune diseases, Blood Transfusion, Multiple myeloma, Haemophilia, Heat treated specimens, Lipemic serum, Anti-nuclear antibodies, T-cell leukocyte antigen antibodies, Epstein Barr virus, HLA antibodies and other retroviruses

*** End Of Report ***

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6. It is presumed that the tests performed are, on the specimen / sample being to the patient named or identified and the verifications of particulars have been confirmed by the patient or his / her representative at the point of generation of said specimen
7. The reported results are restricted to the given specimen only. Results may vary from lab to lab and from time to time for the same parameter for the same patient (within subject biological variation).
8. The patient details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies
9. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.
10. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only



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